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Comusichiamo. First Language, Life-Soundtracks and Storytelling to Support the Cultural Wellbeing of Migrants Living with Dementia

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ABSTRACT
This study, located at the intersection of anthropology, ageing, music therapy and dementia, explores the value of music, life-storytelling and first language, to support the cultural identity and wellbeing of people from culturally and linguistically diverse backgrounds living with dementia in Australia. It uses participant observation, the collection of oral histories and 'life soundtracks' to co-create a song with older Italian migrants and their families. This paper presents the first case study which informed the development of the music engagement program we call *Comusichiamo* (let's make music together). The case study shows that the co-creation of a culturally tailored song, composed and sung in first language, can enhance physical and social engagement, and contribute to the general wellbeing of participants living with dementia, and their carers. Central to the program is the sharing of life history, comprising autobiographic memory, the use of *ipsissima verba*, the precise words participants employ repeatedly, and *ipsissima fabula*, the particularistic stories participants narrate frequently. This music engagement program supports a relationship-centred approach to dementia care, drawing on narrative theory and anthropological notions of personhood, and Benenzon's notion of 'sound identity', to support cultural safety, protect personhood, and guard against 'social death'.

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CaLD; dementia; first-language; music engagement; ageing; life-history

Background and Significance of the Study

In this paper, we present the first case study that informed the development of our *Comusichiamo* music engagement program involving the co-creation, together with participants, of culturally tailored songs in first language, developed through participant observation, including collecting life histories and 'life soundtracks'. Informed by narrative theory (Maggio 2014) and anthropological notions of personhood (Hydén 2011),

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central to the program is the use of *ipsissima verba* – the precise words participants employ repeatedly, and *ipsissima fabula* – the particularistic stories participants narrate frequently. This program was developed in response to the critical gap in services designed for migrants with dementia, particularly in relation to providing culturally appropriate care. Drawing on the anthropology of migration and music-engagement studies, we develop a program methodology that supports a relationship-centred approach, actively involving the person living with dementia and their caregivers. This approach demonstrates the relevance of inter-embodied selfhood within caregiving/care-receiving relationships (Watson 2019), which is particularly relevant to dementia care. We employ the notion of 'sound identity' (Benenzon 1998, 2007) and anthropological notions of corporeality (Fuchs 2020) to emphasise the cultural identity dimension of wellbeing to support cultural safety, protect personhood and safeguard against social death.

We are particularly interested in the role of these programs in counteracting the common experience among those living with dementia of 'social death', defined as a series of losses; loss of social identity, loss of social connectedness, as well as losses associated with disintegration of the body (Králová 2015), and loss of the ability to participate in daily activities (Borgstrom 2017). Such losses often result in an individual becoming disconnected from social life. In addition, the experience of social death can be ascribed by others, specifically when people are considered unworthy of social participation. For example, Brannelly (2011) and Bradshaw et al. (2022) describe this as the ways in which someone is treated as if they were dead or non-existent.

Dementia Care for Migrants

Dementia has become an epidemic (Warren et al. 2021). In 2021, an estimated 472,000 Australians were living with dementia, with the number projected to increase to 1,076,000 by 2058 (Temple et al. 2022). As one in three people over the age of 65 in Australia was born overseas, those with dementia include people from culturally and linguistically diverse (CaLD) backgrounds.[1] Recent literature (Temple et al. 2022) predicts that the number of people from CaLD backgrounds with dementia will increase from about 134,00 in 2016 to 379,000 by 2051. Among them, Italian migrants are one of the largest groups (Baldassar et al. 2006), as most arrived during the massive post-World War II migration intake. Figures for 2021 show that 65.5 per cent of Australians born in Italy were 65 years or older, compared with, for example, 33.1 per cent of those born in England (Temple et al. 2022).

Truswell's (2020) study on migrants in English-speaking countries, including Australia, shows that migrants are at a higher risk of developing dementia due to diverse factors including living outside their place of origin, a common 'retrogression' to their mother tongue and cultural past, and greater rates of social isolation (often due to reduced English language skills). Additionally, Zhao et al. (2021) demonstrate how dementia significantly negatively impacts CaLD individuals' wellbeing triggering loneliness, depression and anxiety. The literature also shows that older adults CaLD Australians, even those that are not living with dementia, are at a higher risk of experiencing poorer quality of life (Jatrana et al. 2018) and higher levels of loneliness, which often lead to depression and anxiety (Lin et al. 2016).

Existing aged care services in Australia do not sufficiently address the needs of Australia's ethnically and culturally diversity populace (Low et al. 2019). Brijnath et al. (2020) claim that there is a lack of cultural and social support, limited resources and limited training on how to address ethnic and cultural diversity. Such omissions are deleterious as they perpetuate social inequalities, neglect older CaLD people and result in poorer individual health outcomes, including increased symptoms of dementia, anxiety and depression (Brijnath et al. 2020). The recent Australian Royal Commission into Aged Care Quality and Standards Final Report (2021) highlights the crucial need to ensure 'cultural safety', defined as:

> … understanding culture and recognising diversity and states that the existing Australian aged care system is not well equipped to provide care that is non-discriminatory and appropriate for people's identity and experience … that aged care providers that do not provide culturally safe care, that is, care that acknowledges, respects and values people's diverse needs … and that staff are often poorly trained in culturally safe practices, with little understanding of the additional needs of people from diverse backgrounds. [Final Report, 2021, 67]

While cultural safety is recognised as a critical concern for First Nations people, there is a notable gap in acknowledging it as an issue for culturally and linguistically diverse (CaLD) communities. In this paper, we explore its relevance to the care of people from migrant backgrounds living with dementia. How exactly 'cultural safety' should be delivered is not well defined in the report, but we highlight it here as our work is about examining what safeguarding cultural identity for migrant populations might involve, and our *Comusichiamo* program is presented as an example. As there is a growing number of studies that explore culturally appropriate services and interventions for individuals from migrant backgrounds living with dementia, this study addresses a critical gap in understanding dementia, migration and wellbeing by examining the experiences of older Italian migrants living in Australia.

Considerations on Musical Approaches for Dementia

Alongside growing attention to the medical treatment of dementia over the past 30 years, there has been increasing interest in non-medical and person-centred approaches, with an emphasis on self-expression, creativity, enjoyment and social inclusion (Ahessy 2017). These social interventions have been utilised to improve the wellbeing and self-esteem of people with mental illness in aged care (Davies et al. 2015), to minimise depression and promote dignity for positive ageing (Greaves and Farbus 2006; Parkinson 2006). Arts engagement, for example, has been demonstrated to considerably improve patients' quality of life (Bradt et al. 2015), generate relaxation by reducing stress and minimise anxiety and depression while increasing empathy and prosocial behaviour (Beard 2012; Davies et al. 2015). Similarly, current innovative research demonstrates the effectiveness of befriending in providing social support to older adults, enhancing their quality of life and mitigating depression and anxiety (Doyle et al. 2021). Additionally, participation in arts engagement projects has been shown to promote psychological wellbeing, positive identity and resilience for caregivers of people with dementia (Hunt et al. 2018), who have been described as the 'invisible second patients' because of the

significant care burden they shoulder (Hallarman 2021). This care burden is often greater for people from migrant backgrounds who have to contend with the additional complexities of language and cultural barriers (Gilbert et al. 2022).

Music engagement and therapy, in the arena of arts-based practice programs, is playing an increasingly pivotal role as a psycho-social intervention for dementia (Gold et al. 2019; Camerlynck et al. 2021). Gerdner was the first to systematically investigate the use of individualised music as an intervention for persons with dementia. It has been proven that music can increase the mood, alertness and engagement of people with dementia, as well as reduce agitation, depression and anxiety (Ahessy 2017). Music benefits individuals by affecting arousal and entraining neuronal activity stimulating emotions, psychological and physiological responses (Harvey 2017). In many cases, people with dementia can remember the melodies and lyrics that have formed the 'soundtrack of their life' (Feld 1984; Owens 2014), because the regions of the brain that process music may be active even in late-stage dementia (Gold et al. 2019; Falcon et al. 2022). These positive outcomes of music engagement can also lead to a reduction in the use of medication, contributing to ameliorating the overall quality of life of people living with dementia, and by implication, also of their caregivers (Harvey 2017).

A music therapy approach particularly relevant to our research is that of therapeutic songwriting (TSW), which involves the creation of lyrics and the use of music within the context of a therapeutic relationship to address a range of therapeutic needs. Baker's (2016) 'Fill-in-the-blank' (FITB) is a method involving the songwriter rewriting some or all the lyrics to a pre-composed song. Studies with analogous approaches demonstrate that the element of songwriting is effective in stimulating cognitive, physical, emotional and social responses, as well as memory recall in people with dementia (Clark et al. 2020). Caneva's (2007) book, '*Songwriting*' offers a detailed account of this type of songwriting process applied with individuals with different medical/cognitive conditions, including children, and illustrates intervention strategies for music therapists.

Surprisingly, there appear to be a limited number of studies that explore the link between music and cultural identity, and very few studies that examine the link between music and first language. For example, music therapist and psychiatrist Benenzon (1998, 2007) developed his interventions to facilitate what he calls a 'sound identity, which he defines as 'a way of walking the path so that the time and space of forgotten elements of our personality may be identified' (Benenzon 2007: 149). Although among Benenzon's (2007) patients there are some with Alzheimer's, his intervention is in a predominantly non-verbal frame, involving mainly individuals with non-verbal autism or patients in a coma. This approach resonates with what Scardovelli (1992) calls a 'sound dialogue', a specific technique of music therapy where two or more people communicate with each other through sounds produced with their bodies, voices, any objects, or musical instruments. Communication occurs through improvisation, mutual listening and each person's reaction to the other's messages. Through this sound dialogue process, Benenzon supports participants to develop their own 'individual sound identity', defined as the sounds, movements, smells, colours, emotions, temperature, touch, pauses, silences and other non-recognizable perceptions, specific to each individual. Facilitating participants to develop their Individual Sound Identity allows them to strengthen self-esteem, recovering personal identity via a cathartic process. The resultant individual sound identity that emerges encompasses ethnic, cultural, family and personal

dimensions. His work, which underscores the individuality and 'essence' of each person, implicitly suggests that music-based interventions may contribute to the preservation of cultural identity (Benenzon 1998, 2007). However, the cultural dimension is under-explored in his work.

In this paper, we build on the work of Benenzon and others to explore and develop the connection between music and cultural identity. To do this, we draw on anthropological understandings of the role of music and storytelling to our cultural identities. By focusing anthropologically on the social dimension of human experience, the study takes into account the role of sound identities, corporeality and the body, and considers the person living with dementia, along with their family and caregivers, as social-relational cultural beings who co-narrate their identities. We believe this approach is needed to develop culturally appropriate interventions that safeguard cultural identity to enhance the quality of life of people living with dementia and their families from migrant backgrounds.

The Contribution of Anthropology: Music, First Language, Identity and Dementia

In anthropological approaches, which focus on the holistic study of shared cultural practices, political, economic and belief systems, including language and folklore (Marino 2020a, 2022), music has been shown to be culturally strategic in identity construction (Carpitella 1974). For example, in migration studies, 'homeland' music has been shown to play a crucial role in releasing stress and nostalgia and assisting migrant groups and individuals to cathartically (re-)affirm their ethnic presence, cultural identity and memory, and to reconnect socially in the 'hostland' (Marino 2012). In this context of migration, the expression or performance of an individual's, or group's, ethnic presence represents their 'ontological being' and has the capacity to protect cultural identities particularly in situations and contexts where they experience a sense of de-personalisation and dislocation, including feeling out of place, either physically (i.e. migration) or socially, including experiences of 'social death' (De Martino 1977). Lévi-Strauss (1966) anticipated these notions by asserting that music not only elicits emotions, but also the habitual words, idioms and stories – that we call *ipsissima verba* and *ipsissima fabula* – shared and performed within a group, which play a critical role in protecting against an 'identity crisis' by reinforcing the sense of self.

The value of anthropology in understanding dementia can also be seen in its socio-relational approach, and in particular its appreciation of the role of narration and corporeality. Narrative theory, and the importance of narration in human society, is a central area of study in anthropology. The relevance of sharing memories through story telling has been discussed for decades (Fabietti 2010; Bönisch-Brednich et al. 2023). Fisher (1984) suggested the term *homo narrans* (instead of *homo sapiens*) as a more applicable in defining 'our' species. According to Niles (1999), narration is the *raison d'être* of human existence. Similarly, Fabietti (2010) argues that narrating is a cultural practice central to the nurturing of an individual's sense of self. Narrative theory argues that we narrate our own, and each other's histories, in ways that create, maintain and sustain our socio-relational worlds (Fassò 2007). The purpose of narrating collective and autobiographical memories not only includes cultural transmission and the

strengthening – or co-constructing – of cultural and social belonging, but also plays a functional role in seeking consensus, and validating the individual and collective social identity (Halbwachs 1925; Durkheim 1973). It is via the sharing – through narration – of the individual or the group's memory, what Halbwachs (1925) refers to as *les cadres sociaux de la mémoire* (social frameworks of memory, or social memory) that affirm our identities, both as individuals and collectives. Research shows that a social environment that facilitates the personhood of dementia patients, through regular acts of communication and interaction, has a positive effect on health and wellbeing and the ability to remain socially engaged, even if the person has a limited capacity to respond (e.g. Hydén 2011). Carers and cared-for can create a 'nurturative relational context' (Hellström et al. 2005) to sustain the quality of their relationship.

Relatedly, anthropological notions of corporeality and the body are particularly relevant in the study of dementia as they highlight the phenomenology of embodiment that rejects a dualistic and dichotomising framework, in which the body is regarded as a mere vehicle of the individual, or a carrier of the brain as the organ of mental faculties (Fuchs 2020). In this approach, human personhood is understood to be defined by relational sociality, which manifests itself in the primary, pre-reflective intersubjectivity of early childhood and which is crucially based on intercorporeality and interaffectivity (Fuchs 2020). In other words, and following Fuchs (2020), people do exist, even if the *res cogitans* – 'that thinking thing' – does not appear to think, as individuals have 'body memory', not merely a cognitive representational memory. Body memory, which involves the embodiment and enactment of familiar habits, practices and preferences (Kontos 2004), has a potency of its own, comprising implicit and unconscious knowing. This approach is especially relevant to addressing the experience of social death. It challenges conventional conceptualisations of dementia as 'the erosion of selfhood', including Cartesian divisions between mind and body. For example, socio-personhood approaches emphasise the relational aspects of the self that remain intact despite cognitive impairment (Twigg and Buse 2013). This approach highlights how selfhood continues to be enacted through praxis, habit and habitual, embodied practices and gestures (Kontos 2004), including through an individual's music engagement habits.

Drawing on these corporeal and relational understandings of personhood and identity, music and first language, we argue that bodily memory can be stimulated through practices that support individuals to be at 'home in the world'. This is in line with Merleau-Ponty's (1962) logic suggesting that performances of life enter into the memory of the body and remain preserved as dispositions and potentialities. As Merleau-Ponty (1962: 148) pointed out, 'the body is solidified existence and, for its part, existence [is] perpetual incarnation'. We agree with Merleau-Ponty that this history of the body holds a key to ways to bring back the person to be at home in the world, and the dementia body is especially relevant in this regard.

Further supporting the relational and corporeal understandings of being and personhood, anthropological methodologies aim to give back the other's voice, and value deep engagement and participation. It is via the grounded theory insights that are delivered through participant observation, during ethnographic fieldwork that the anthropologist observes and interacts with individuals performing their cultural practices and constructing and reconstructing their identities (Malinowski 1922; Marino and Chiro 2014;

Marino 2020b). Individuals, in this arena, are seen as social actors; rather than 'body-objects'. They are 'experiencing body-subjects' who co-share and co-construct meanings (Sheets-Johnstone 2020). In line with (and expanding on) Gibson et al.'s (2020) criticism of the soft paternalism of healthcare, we argue that some psychological and allied health approaches can be limited by a person-centre care approach, which tends to focus on the individual, rather than recognise that the individual is also a socio-relational being, corporeally immersed in relationships with others. The Senses Framework offers a promising alternative with its emphasis on relationality, though it could benefit from a stronger focus on cultural identity.[2] In the case of dementia patients, their relationality and corporeality are at risk of becoming invisible through social death, and their intersubjectivity and intercorporeality, in particular with family members, but also with paid carers, is critical to safeguarding their cultural personhood.

Study Design and Methodology

This paper focuses on Giuseppe's case[3], which informed the development of the music engagement program we call *Comusichiamo*.

The first stage of analysis involved reviewing the participant – Giuseppe's – oral history and life-soundtrack collected by Marino in 2014, about his migration experience and, in particular, his ethnic identity. That interview was part of Marino's PhD research, a larger study involving ethnographic research among families within the Italian community of Adelaide, most of whom, like Giuseppe, originated from Calabria, in the south of Italy (Marino 2019, 2020a). To conduct that research, Marino situated himself in this community as both a researcher and as a folkloric musician of Calabrian music – born in Rome, with a family originating from Calabria, Marino speaks Calabrian dialect, and is intimately familiar with its culture.

The second stage of analysis involved reviewing the observation notes Marino collected over a series of 10 meetings with Giuseppe in the aged care facility where he lived, comprising 2 visits, one week apart, before the pandemic lockdown in November 2019, and 8 weekly visits after the lockdown ended from July to August 2021. These visits were conducted by Marino in his role as unpaid volunteer visitor in the facility. Discussions between the authors in this period revolved around how to support the social engagement of residents during the lockdown, in particular those living with dementia. The idea developed of composing a song for Giuseppe, based on the oral history and life-soundtrack Marino collected in 2014. During the 8 weekly visits post-lockdown, Marino performed the song for Giuseppe and aged care staff.

Stage three involved analysis of the data from stages one and two in light of the research question that emerged from the data: 'To what extent can culturally tailored songs (including the process of co-composing) trigger cultural memory, facilitate physical and social engagement and mental activity, and improve wellbeing for people living with dementia from culturally and linguistically diverse backgrounds?' Analysis involved grounded theory building from the data (Glaser et al. 1968; Kvale and Brinkmann 2009). The grounded theory approach aims to highlight key aspects of the participant's lived experiences, while the researcher attempts to generate theory from the interpretation of the data. Following this logic, '… analytic understandings are discovered in the sense that they emerge in large part from detailed examination of observational

fieldnotes, and are then tested and revised in a constant comparative process' (Kvale and Brinkmann 2009: 106). This grounded theory analysis, drawing on individual sound identities, narrative theory and anthropological notions of corporeality led to the findings about the value of sharing of first language, life history and life soundtrack, comprising autobiographic memory, the use of *ipsissima verba* – and *ipsissima fabula* – to safeguard personhood and cultural identity.

Giuseppe's Case Study

In 2019, Giuseppe was a widower in his late 80s. He migrated to Australia when he was 18 years old from the Calabrian village of Paese Sopra, a small rural village situated 60 kilometres from the city of Reggio Calabria. Marino first met Giuseppe in 2014, at a time when Giuseppe was regularly active within the Italian community of Adelaide. In his fieldnotes from meetings with Giuseppe, Marino records that he attended numerous *feste* (community celebrations) and enjoyed listening to and dancing the *tarantella*. In 2019, Giuseppe's *compare* (family friend) died. Soon after, he was hospitalised for two weeks due to a pulmonary infection. He was diagnosed with dementia and transferred to an aged care facility. When Marino went to visit him there for the first time, Giuseppe did not recognise him.

The following are extracts from Marino's observation notes, visiting him in the residential care facility in November 2019.

> […] I see Giuseppe sitting on a chair, together with a small group of residents (six women and two men), watching TV. Giuseppe is the only Italian, except for a lady who is speaking in English with another woman sitting next to her.
>
> A nurse sees me with him, and comes straight up to us, and says: 'Giuseppe, your friend is here, are you happy?'
>
> Giuseppe does not answer, and the nurse says: 'Giuseppe does not speak English, and no one here speaks Italian'.
>
> I tell the nurse that Giuseppe's English is reasonably good, as he came to Australia when he was young. The nurse shrugs her shoulders and leaves.
>
> Since it is lunchtime, I leave the facility. […]

Not only is Giuseppe not receiving any visitors, but Marino also notices that there are no staff who can speak Italian. One week later, he returns; the following is an extract from his observation notes:

> […] I see Giuseppe sitting in a wheelchair, with the same group, in front of the TV. No one is talking; some residents are asleep in their chairs.
>
> Although Giuseppe is looking at the TV screen, his gaze is absent.
>
> I want to interact with him. As I approach him, close enough to touch his wheelchair, he looks very intimidated. He starts crying.
>
> A nurse intervenes and takes us to his room.
>
> I take a seat (a little further away); with a comforting voice, I ask him (in English) how he is. He does not respond. He is looking at me with frightened eyes.

> As soon as the nurse leaves his room, I begin to use Calabrian language, saying: '*Don*[4] *Peppinu, Comu iamu*?' [Mr Giuseppe how are we going?]. I am using the title *Don* to bring something familiar into his world; the title *Don* is appreciated by several older Calabrians.
>
> I am now talking in Calabrian. Although he does not respond, I notice he appears interested.
>
> I mention his village, Paese Sopra. Giuseppe starts crying. Then he says (in Calabrian) 'no one visits me'. […]

Due to Covid-19 restrictions, Marino was unable to visit Giuseppe for several months after this second visit. In the absence of access to the participant, Marino drew on Giuseppe's oral and migration history and life-soundtrack, the information about his life that he had collected in his previous study in 2014. This is an extract from Marino's 2014 fieldnotes:

> I am at Giuseppe's house […] in the living room there are pictures of his village (Paese Sopra), I see the emblematic Norman castle, built on 'the Rock', known as *u casteddu supa a Rocca* [the castle on the rock], which is the symbol of his village.
>
> Giuseppe is telling me that it was near that castle that he was kicked by a donkey, fastened by a rope to a pole, when he was about 8 years old, after he had pulled the donkey's tail. [This is a story he often repeats to me].
>
> He shows me photos of the church of San Sebastiano, located in *a chiazza* (the main square of the village).
>
> It was there, he says, that he used to play with his best friends, Carmelo and Umberto, when he was a child.
>
> […]
>
> He is saying it was there that every Sunday they would see Don Pietro, the *mafioso* of the village, walking out of the church, well dressed, greeting everyone, including them.
>
> I am noticing how entertaining he is and how much he is enjoying telling stories, and how proud he is of his Italian background.
>
> […]
>
> He is making a joke in response to my asking him in Calabrian '*Allura*?' [colloquially: What are you up to?], with his gag response '*60 minuti*' [sixty minutes] while showing me his wrist – the Calabrian word *allura* sounds like the Italian *all'ora* [per hour].
>
> […]
>
> After joking around, Giuseppe pretends we are boxing; he is keeping his guard high. Such fighting pose is a 'joke'. This seems a pretext to start recalling the hard times, since he is saying he had to fight against the '*cancaruni 'mbriachi*' [drunk Skippies[5]], who were looking for trouble and called him names such as 'dago'. [This is another story he often repeats]
>
> This happened in the 1960s, when he spent several months in northern Queensland, working on constructing the local railway.
>
> On that matter, he often says: 'My life was mainly cutting canes and killing snakes'. […] [This is a common refrain he repeats regularly]

Drawing on this earlier interview data with Giuseppe, Marino composed the lyrics of a song based on Giuseppe's 'life-story'. In the lyrics, Marino included his name, the name of his hometown (Paese Sopra), the names of his friends from childhood (Carmelo and Umberto), some relevant places, his job in Australia (working for a railway company). Particular care was taken to include the '*ipsissima verba*' and '*ipsissima fabula*', the precise words and particularistic stories that Giuseppe often repeated in his dialogue.

In terms of the genre of the music, Marino chose to compose a *tarantella*, a folkloric tune frequently played in Calabria, which was familiar to Giuseppe, and which Marino knew from Giuseppe's life-soundtrack that he enjoyed listening and dancing to. The tarantella was regularly played at the family and community events Giuseppe attended throughout his life in both Italy and Australia. Thus, this melody has been known to Giuseppe from his early childhood and has been enjoyed regularly by him ever since. The resulting song, '*Giuseppe's tarantella*' was recorded and saved on a CD and in MP3 format, so that Giuseppe would be able to listen to it at any time.

After Covid-19 restrictions were eased in July 2021, Marino returned to the facility to play this tailored song live to Giuseppe every week, for eight weeks. Further entries from observation notes report Giuseppe's' response to Marino's subsequent visits:

> The first visit, when the introduction to the song began, Giuseppe did not show any interest. However, when the singing (in Calabrian) started, he bent his body forward, and looked at me with wide open eyes. When Giuseppe heard his friends' names, he started to cry.

Over the series of 8 weekly visits, Giuseppe began to display an increased interest in Marino's musical instrument, and it was visibly clear that he enjoyed listening to the song. He also looked more engaged during the meetings, with body movement and facial expressions. Marino records in this notes that 'A nurse commented that Giuseppe was brought back to life'. Further entries from the observation notes report Giuseppe's response to Marino's subsequent visits:

> The fifth week I returned to the facility to meet Giuseppe, and at the end of the song, I asked him if he liked it. He answered: '*È bella*! *Bravu*' [It's nice! Well done]. Giuseppe was displaying his emotional response in the moment. I asked him if he wanted to listen to the song again. He said yes. This second time, his body rocked gently, rhythmically, with the music. He cried again.
>
> After listening to the song twice, I started asking Giuseppe about his hometown, his friends, and his job experiences in Australia. He answered with a few words. He also imitated the fighting pose. Then he pointed out my beard, making the gesture of shaving and making the sound of a goat. After we concluded the music session, when we were in the common area, another resident stopped in front of Giuseppe and said, in English, 'He's good man'. Giuseppe answered in Calabrian: 'Mind your *fucking* business'. He then told me that people were not friendly there, and asked me if I could stay for dinner and when I would visit him again.

In the final week of visits, Marino gifted Giuseppe the CD of the song, including the lyrics in Italian language and English. Building on the experience of working with Giuseppe, the authors have designed a music engagement pilot program called *Comusichiamo*.

Data Analysis

Comusichiamo involves the co-composition of a culturally tailored song using a melody from the 'soundtrack' of the participant's life that is sung in their first language, featuring the *ipsissima verba* and *ipsissima fabula* that they and their family and carers use to narrate their lives. In the discussion below we analyse the case study by exploring the role of music and first language in safeguarding cultural identity, drawing on narrative theory and anthropological conceptions of corporeality, as well as Benenzon's notion of individual sound identity.

The choice of song was informed by the participant's life history, including the melodies, rhythms and language central to his cultural identity. Giuseppe's song is a tarantella, the most common folkloric melody relevant to his hometown and childhood memories (Marino 2012). This melody, sung in Calabrian dialect, helped to stimulate and reinforce Giuseppe's sense of belonging, reassuring him that he is part of his community – both of the past (i.e. Italy, Paese Sopra) and present (Nolan et al. 2006). In addition, Giuseppe's personal biography, including his story, skills, duties and other relevant knowledge of the past, were featured in the words of the song, further strengthening his connection to his cultural identity. The song also touched upon the goals he valued and aspired to, such as working hard and saving money, and featured purposeful activities he frequently engaged in during his past, like playing with his best friends.

Featured in the words of the song, and central to its impact, is the use of *ipsissima verba* (the precise words Giuseppe employed frequently) and *ipsissima fabula* (the particularistic stories he narrated frequently). These words and phrases were carefully selected from the life history narrative in the first language. The use of these personalised lyrics resulted in visible temporary changes in Giuseppe. Listening to his song contributed to nurturing a sense of achievement. The resultant individual sound identity is deeply rooted in the embodied and corporeal experiences that the music evokes, creating a profound connection between past and present.

The positive changes observed in Giuseppe following his engagement with his personalised tarantella song can be interpreted through the established effects of music on individuals with dementia. When Giuseppe listened to his song, the familiar melody likely evoked a nostalgic recall of autobiographical memories, eliciting not just emotional responses but also physiological reactions (Harvey 2017). The literature suggests that this interaction with music and language engages brain regions responsible for processing familiar tunes and language, which often remain functional even in late-stage dementia (Falcon et al. 2022).

Moving to an analysis of Giuseppe's bodily responses, using the body as the analytical lens (Watson 2019), highlights what Matthews (2012) and Kontos (2004) refer to as the bodyliness, or corporeality, of human beings. Listening to the song elicited movements in Giuseppe's body, including gentle rocking back and forth, facial expressions (smiles and frowns), eye behaviour (blinking, winking, direction and length of gaze and pupil dilation) and posture, which exemplify what Zeiler (2013) calls the dynamics of intercorporeal engagement in dementia. During the 8 weekly meetings of stage two, Giuseppe demonstrated an apparent improvement of engagement in his body movements, including facial expressions, and eye behaviour after he listened to his song.

In addition to bodily movement, Marino's notes from these meetings detail an improvement in Giuseppe's communication after he listened to his song, in that he answered questions about his story. Giuseppe showed changes in improved mood, increased engagement and interest, especially in the moment. Not only did he display an increased interest in the musical instrument and seemed to enjoy listening to his song, but he also displayed emotions, including crying, nodding and smiling. Giuseppe showed a positive response to listening to his personalised story and was particularly animated in the final week when a copy of the song on CD was gifted to him. Marino noted; 'He also looked like his 'old self'; his sense of humour came back, as he made those jokes that he used to make in the past'.

As Giuseppe had few visitors, the authors requested that the care provider continue to play the CD and include this in his care plan. This aligns with Juul et al. (2019), who emphasise the necessity of obtaining management approval for interventions to ensure their sustainability, particularly given the limited provision for social care and the need for such measures to be formally incorporated into care plans (Baldassar and Wilding 2020).

Discussion and Conclusion

Giuseppe's case study has informed the development of a music engagement program we call *Comusichiamo*, that features the careful collection of the *ipsissima verba* – the precise words participants employ repeatedly, and *ipsissima fabula* – the particularistic stories participants narrate frequently, through anthropological oral history and migration story telling. This process of co-composition using these oft-repeated phrases reflects the therapeutic songwriting method (TSM), which utilises the participant's own words – usually garnered through therapy prior to lyric and music creation. In comparison to TSM, we draw on participants' migration life histories, with a particular focus on crafting personalised lyrics using their *ipsissima verba* and *ipsissima fabula.* We note that this songwriting method process is possible even when drawing on previous in-depth interviews, as in Giuseppe's case-study, which makes it particularly suited to working with people with dementia who may have limited capacity for memory and speech. The individual with dementia is at risk of social death, including loss of social and cultural identity, given that they often struggle to participate in the process of co-narration on their own. Because they cannot dialogue with others in socially reliable and recognisable ways, others must intervene on their behalf, and the *Comusichiamo* program facilitates this process.

In addition to the *ipsissima verba* and *fabula*, we found that among the key elements of life history and migration stories that trigger memory, elicit increased engagement, and affirm cultural identity are the names of key people, family and friends, their places of origin, travel and destinations, their jobs and work experiences, married life and children, their aspirations and achievements, and major life events, like meeting a life-partner, marriage and the birth of children. Particularly significant is the recounting of all these words and stories in the first language, which functions to stimulate memory, encourage subsequent engagement and facilitate interactions (Clemente 2013). Crafting these elements into a song that uses the genre of music featured in the participant's life soundtrack further enhances their capacity to engage. This approach aims to focus on

safeguarding and affirming cultural identity, exploring the cultural dimensions of their life and involving them in a relational, creative and humanising process.

Although not a feature of Giuseppe's case study, focusing on the individual's *ipsissima verba* and *fabula* facilitates the collective co-narration of personhood and involves carers who are often deeply familiar with these oft-repeated phrases and stories used by the participant. The co-narration process highlights the critical role of carers in supporting individuals living with dementia to share their stories, thereby reaffirming their socio-cultural and relational personhood and safeguarding them from social death. This co-creation process also enhances carers' ability to interact effectively with the person living with dementia, reflecting a relationship-centred approach to care.

While there is much to recommend a person-centred care approach, which focuses on the specific needs of the individual, a relationship-centred care approach considers the individual – with (or without) dementia – as a social-relational cultural being. Here, anthropological approaches are particularly useful, as they feature individuals as relational animals suspended in webs of significance (Geertz 1973) whose personhood comes into being via interactions, stories and relationships. It is this relational element of the *Comusichiamo* program, defined through the body, music, first language culture and its semiotics, that can play an innovative role in engaging and supporting individuals with dementia. We believe that featuring the repeated words of *ipsissima verba* and the repeated stories of *ipsissima fabula*, provide a bridge to an individual's relational narrative histories creating an emotional connection to a meaningful past. It is the relational aspect that keeps the person alive as a subject (not object). Following such an anthropological logic, the *Comusichiamo* approach affords people living with dementia an 'agential role (in) the care assemblage' (Jenkins 2014), which helps challenge the experience of social death. Their agency is facilitated through the *Comusichiamo* process by enabling people to come together to create the conditions in which inter-embodied selfhood can 'spring forth in joint activity' (Zeiler 2013).

Overall, the case study indicates positive changes in Giuseppe, including progress in communication and social engagement, which could be described as a re-activation of his personhood, with particular relevance to his cultural identity. Giuseppe seemed, from an anthropological point of view, to have validated his 'ontological being'; his cultural and social identity, concretised in the song itself. Another important feature of the case study is the time it took for Giuseppe to respond to the song. It was only after several visits that he began to react more to the song. This suggests that such music programs should be long-term. In fact, a systematic review identified that the duration of psychosocial support programs needed to be at least three to six months to have a positive impact on people with dementia and caregivers (Ghosh et al. 2022). Given the progressive deterioration of dementia and the increased need for home care by family members, the authors recommended delivering long-term programs to maintain the positive effects on people with dementia and caregivers (Ghosh et al. 2022).

In this paper, we limited our analysis to one participant. Further research with a larger sample of participants is needed to provide more empirical evidence about the role of music and first language in the cultural wellbeing of participants of migrant backgrounds. In addition, the present case study, although it provides evidence for the efficacy of the practice with Giuseppe, is limited by the circumstances of the pandemic. While it is understood that we cannot generalise based on a single case study, it was Giuseppe's

case that informed the development of the *Comusichiamo* program, and also inspired our ongoing research enquiry.

The case study suggests that the *Comusichiamo* program can be used to enhance meaningful physical and social engagement, provide cultural safety, protect personhood and safeguard against social death, contributing to the general wellbeing of people from migrant backgrounds living with dementia and their family and carers. It is our aim that the *Comusichiamo* program can make visible the dynamics of inter-embodied selfhood and cultural identity to broaden the scope and opportunities for relationships, returning dignity and acting as a counter to the 'social death' often experienced by people living with dementia.

By exploring the relevance of music, migrant storytelling and first language, in particular by featuring the ethnic, family and cultural identities of participants, this paper expands on Benenzon's (1998, 2007) work and contributes new insights to music-engagement interventions by bringing the voices and cultures of those living with dementia to the centre of research and practice (Downs 2013). More specifically, by focusing anthropologically on the social dimension of human experience, the study takes into account the importance of music, first language and the body, and considers the person living with dementia, along with their family and caregivers, as social-relational cultural beings who co-narrate their identities. We believe this approach is needed to develop culturally appropriate interventions that safeguard cultural identity and enhance the quality of life of people living with dementia and their families from migrant backgrounds.

A key aim of the *Comusichiamo* program is to address the critical gap in services specifically designed for people living with dementia from culturally and linguistically diverse backgrounds, and their carers. Additionally, and more specifically, as anthropologists of migration studies, we focussed on the development of a program that addresses the need to safeguard cultural identity and personhood, by featuring social relational approaches to caregiving that recognise people living with dementia as socio-cultural beings whose identities are co-narrated in relationship with their carers and family.

Notes

1. Due to its prevalence in the relevant literature and government reports, we have used the acronym 'CaLD' (culturally and linguistically diverse) in our paper. However, we are sensitive to the limitations of the term as a problematic label that generalises and obscures the unique cultural identities of individuals, as noted by O'Neill (2020) and Scarino et al. (2015). The term not only objectifies people, but it also implies a normative view of diversity that can inadvertently perpetuate institutional racism and reinforce outdated views of Australian society. For this reason, where possible, we prefer to refer more specifically to the participants and individuals in our study.
2. An issue the authors address in a forthcoming paper.
3. All names of places and people are pseudonyms.
4. *Don* is etymologically from Latin *dominus* 'lord'. This title is used in some southern Italian rural areas to show respect to men, usually, but not exclusively, to elders (Marino 2020a).
5. A colloquial term, sometimes used derogatively, for Anglo-Australians.

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